

INTRODUCTION

According to the definition given by the World Health Organization (WHO), generic medicines or generics are pharmaceutical products usually intended to be interchangeable with the innovator product, marketed when period of patent is over or other exclusivity rights. Generic medicines are like boon among people who usually tends to ignore the hazardous implications on their health, just because of the high cost of the branded medicines. That is why it is contributing in improving the healthcare system of the country. People of India feel difficulty in choosing between branded and non-branded products. On the other hand, in western countries there is no confusion between these two because the concept of generic medicines is very well established. United States filled about 89% of all the prescriptions with generic medicines and as far as India is concerned, it is in emerging phase . Generic or branded medicines are not clearly defined under the Drugs & Cosmetics Act, 1940 and Rules, 1945. However, generic medicines are those which contain same active ingredient(s) with same amount in same dosage form, administered by the same route of administration and equivalent safety and efficacy as that of branded medicines . The entry and progress of generics into the market is positive sign for patients especially poor and needy, medical organizations, governments (States/Union territories) and negative for manufacturers of branded medicines

PRINCIPAL

The Jan Aushadhi Scheme (Public Medicine Scheme) is a direct market intervention scheme launched by the Department of Pharmaceuticals, Ministry of Chemicals and Fertilizers, Govt. of India.

- To make available quality generic medicines at affordable prices to all citizens through a special outlet known as Jan Aushadhi Store (JAS) opened in each district of the States. Around 108 such outlets have been in operation as in September 2015.
- Jan Aushadhi stores have been set up to provide generic drugs, which are available at lesser prices but are equivalent in quality and efficacy as expensive branded drugs.

- Department of Pharmaceuticals of Government of India set up by the Department of Pharmaceuticals for implementation of the Jan Aushadhi Scheme in December 2008. It got registered as an independent society in April 2010.

Bureau of Pharma PSUs in India (BPPI) are running Jan Aushadhi Kendras as a part of Pradhan Mantri Bhartiya Janaushadhi Pariyojana. These are the centers from where quality generic medicines are made available to all. 726 districts of the country have Jan Aushadhi Kendras.

The turnover of Janaushadhi Kendras in the country touched around ₹600 crores during 2020-21. The number of kendras stood at 7,500 now. The product basket of the Pradhan Mantri Bhartiya Janaushadhi Pariyojana(PMBJP) has evolved from a mere 131 products in 2014 to 1,449 medicines and 204 surgical items.

What is the role of BPPI in PMBJP?

The Department of Pharmaceuticals of Government of India has established a Bureau of Pharma Public Sector Undertakings of India to support the CPSUs to coordinate the sales, marketing, supply and procurement of the generic medicines. These medicines are distributed to healthcare seekers through Pradhan Mantri Bhartiya Janaushadhi Kendras.

The Bureau of Pharma PSUs of India (BPPI) was appraised for ensuring the supply of medicines and other pharma products like masks to people at affordable rates during Covid-19.

PMBJP OBJECTIVES

The scheme aims at educating the masses about generic medicines and that high prices are not always synonymous with high quality. It intends to cover all therapeutic groups and create demand for generic medicines through medical practitioners.

PMBJP FEATURES

- They can be set up by doctors, pharmacists, entrepreneurs, Self Help Groups, NGOs, Charitable Societies, etc. at any Under PMBJP, **Pradhan Mantri Bhartiya Janaushadhi Kendras (PMBJK)** are set up across the country so as to reduce the out of pocket expenses for health care.
- The Bureau of Pharma PSUs of India (**BPPI**) under the Department of Pharmaceuticals is involved in coordinating, procuring, supplying and marketing generic medicines through PMBJK.
- The procured generic medicines are sold at 50% to 90% lesser prices as compared to the market prices of branded medicines.
- All drugs procured under this scheme are tested for quality assurance at NABL (National Accreditation Board Laboratories) accredited laboratories and is compliant with WHO GMP (World Health Organisation's Good Manufacturing Practices) benchmarks. Government grants of up to 2.5 Lakhs are provided for setting up of PMBJKs.
- suitable place or outside the hospital premises.
- The railway ministry has given in-principle approval to opening up of Jan Aushadi Kendras at railway stations and other railway establishments under PMBJP. This is a significant move, as the access to **generic medicines on railway stations** would boost the accessibility, affordability of essential medicines and improve the convenience for the customers.

What are Generic Medicines?

Generic medicines are unbranded medicines that are equally safe and have the same efficacy as that a branded medicines in terms of their therapeutic value. The prices of generic medicines are much cheaper than their branded equivalent.

Outreach of generic medicines –

- With developments like more and more doctors prescribing generic medicines and the opening of over 6600 Janaushadhi stores across 652 districts, awareness and availability of high-quality affordable generic medicines have increased in the country. About 10-15 lakh people benefit from Janaushadhi medicines per day and the market share of generic medicines has grown over threefold from 2% to 7% in the last 3 years.
- The Janaushadhi medicines have played a big role in bringing down the out-of-pocket expenditure of patients suffering from life-threatening diseases in India. The PMBJP scheme has led to total savings of approximately Rs.1000 crores for common citizens, as these medicines are cheaper by 50% to 90% of the average market price.
- The PMBJP is also providing a good source of self-employment with self-sustainable and regular earnings.

-

. JAN AUSHADI SUGAM APPLICATION

The PMBJP launched a mobile application in order to help people locate the Janaushadhi Kendras in the areas around them.

Since smartphones are seen accessible to all the groups of the society, the PMBJP aims to promote their affordable healthcare scheme through available technology. Affordable healthcare seeker can easily find these PMBJK and get generic medicines at a substantially low cost compared to that of the branded OTC (over-the-counter) ones.

India is one of the highest per capita out-of-pocket expenditures' country, such generics will save a lot of money which can be used for other health issues. In all the countries, use of generic drugs has increased significantly in recent years. In 2008, the Government of India, through the Department of Pharmaceuticals, started a new initiative "Jan Aushadhi" (a Hindi word literally translated as "Medicine for People"). This program envisaged making unbranded quality medicines available to poor people in the country at a reasonable and affordable price through retail outlets' setup with the help of the government. It has taken ownership of setting up Jan Aushadhi stores, which are pharmacies selling only generic name medicines to the extent possible, giving preference to pharmaceutical public sector undertakings too. Until March 15, 2018, 3200 Jan Aushadhi stores were operating in more than 33 states/union territories across India. There are not enough Jan Aushadhi stores, possibly 3200 against more than 8 lakh retail pharmacies in existence, with many rural areas still underserved.

The Medical Council of India, in an amendment to the code of conduct for doctors in October 2016, has recommended that every physician should prescribe drugs with generic names legible and he or she shall ensure that there is a rational prescription which promotes the use of generic drugs. In future, the Government of India may bring a legal framework under which doctors will have to prescribe generic medicines to patients.

QUALITY ISSUES

Generic medicines are typically 30%–80% cheaper than originator equivalents. The question raised quite often is “Whether the quality and performance of generic drugs is comparable to the brand drugs?” The proponents of generic drugs claim that they are equally effective as brand or innovator drugs. Subsequent to this claim, the Drugs Technical Advisory Board of India in May 2016 considered amending Rule 65 (11A) of the Drugs and Cosmetics Act, 1940, so that pharmacists can dispense generic name medicines and/or equivalent brands against prescriptions in brand names. However, skeptics have stated that the use of generic drugs may lead to prolongation of illness or even therapeutic failure as the bioavailable brand.

Hence, the critical issues that affect the quality of generic drugs are purity, potency, stability, and drug release, and these should be controlled within an appropriate limit, range, or distribution to ensure the desired drug quality.

INDIAN SCENARIO

To ensure that the quality of generic drugs is equivalent to its branded version and that this equivalence is scrupulously maintained, the government should also bring legislation to ensure compliance in generic drug manufacturing and testing.

Currently, undergraduate medical students learn the concept of generic drugs during their second MBBS teachings in pharmacology. Beyond this initiation, the importance of generic drugs in reducing patient expenses is not adequately emphasized during their further undergraduate and postgraduate training. Experts who formulate medical education curriculum would have to address this lacuna to ensure that future practicing physicians are well versed and comfortable in prescribing generic drugs.

Pharmacies and drug stores other than in government-run hospitals and clinics hardly stock unbranded generic drugs. When a generic drug is

prescribed, it is likely that the pharmacist would dispense his favorite branded generic. Unless the prescribers are sure about the quality of generic drugs, compulsory prescribing in generics holds no breath in it.[10] Central Drugs Standard Control Organization is a licensing authority in India which has recently released guidelines for BA/BE studies.[11] It has given a checklist for submission of documents before undertaking BA/BE study, but is not clear whether they will ensure bioequivalence of each and every molecule. Confidence and habit of generic prescribing need to be inculcated into our medical fraternity by providing information and education particularly in areas of bioequivalence, regulatory aspects, and dispelling myths, fears, doubts, or false ideas about generic medicines. Only providing guidelines will not enhance quality drugs in market and prescribing. On the other hand, ensuring quality with strict regulatory mandate and providing updated information regarding the generic drugs (as given by United States Food and Drug Administration in its Orange Book) will eventually enhance prescription of generic drugs. The Orange Book provides information of approved drug products with therapeutic equivalence evaluations on a monthly basis. It serves as public information to state health agencies, prescribers, and pharmacists to promote public education in the area of drug product selection.

Healthcare expenditure continues to rise in countries where healthcare is not a priority. Healthcare providers and policymakers need to keep drug prices affordable. Generic drugs offer an important tool for reducing the overall healthcare expenditure. As more and more patents expire, the generic versions of the pharmaceutical market are expected to continue for increased sales. A sample list of generic drugs available in India along with their indication is depicted in Table.

Cost price of a few generic drugs

Name of the generic drug	Indication	Price
Paracetamol	Fever	Rs. 9.80/10 tablets
Cefixime	Bacterial infection	Rs. 225/10 tablets
Amoxicillin	Bacterial infection	Rs. 30.77/10 tablets
Ofloxacin	Diarrhea	Rs. 92/10 tablets

With the availability and usage of generic drugs, the experience with these drugs will increase and it will be possible to compare the real-time effectiveness of generic drugs vis-à-vis brand drugs.

Only after real-time effectiveness of generic drugs is documented and made published, will physicians be able to easily convince patients, including the wealthy, to take generic drugs. Our government can achieve this goal, if the regulator maintain high standards for safety, efficacy, and quality in the review of generic drugs.

If sponsors, healthcare providers, and regulators work together effectively, then effective use of generic drugs will make medical costs cheaper and also ensure sound treatment options for the patients.

